and salts creates osmotic gradient in the medulla. The osmotic gradient is also due to the permeability of the collecting duct to urea.

The vasa recta, maintains the medullary osmotic gradient via counter current exchanger (the flow of blood through the ascending and descending vasa recta blood vessels) by passive transport. Figure 8.8 (b) shows counter current exchanger where the vasa recta preserves the medullary gradient while removing reabsorbed water and solutes. **This system does not produce an osmotic gradient, but protects the medulla** by removal of excess salts from the interstitial fluid and removing reabsorbed water. The vasa recta leave the kidney at the junction between the cortex and medulla. The interstitial fluid at this point is iso-osmotic to the blood.

When the blood leaves the efferent arteriole and enters vasa recta the osmolarity in the medulla increases (1200mOsm) and results in passive uptake of solutes and loss of water in descending vasa recta. As the blood enters the cortex, the osmolarity in the blood decreases (300mOsm) and the blood loses solutes and gains water.

At the final stage in collecting duct to form concentrated urine (hypertonic). Human kidneys can produce urine nearly four times concentrated than the initial filtrate formed.

List the pathways involved in the homeostatic compensation in case of severe dehydration.

8.4 Regulation of kidney function

ADH and Diabetes insipidus

The functioning of kidneys is efficiently monitored and regulated by hormonal feedback control mechanism involving the hypothalamus, juxta glomerular

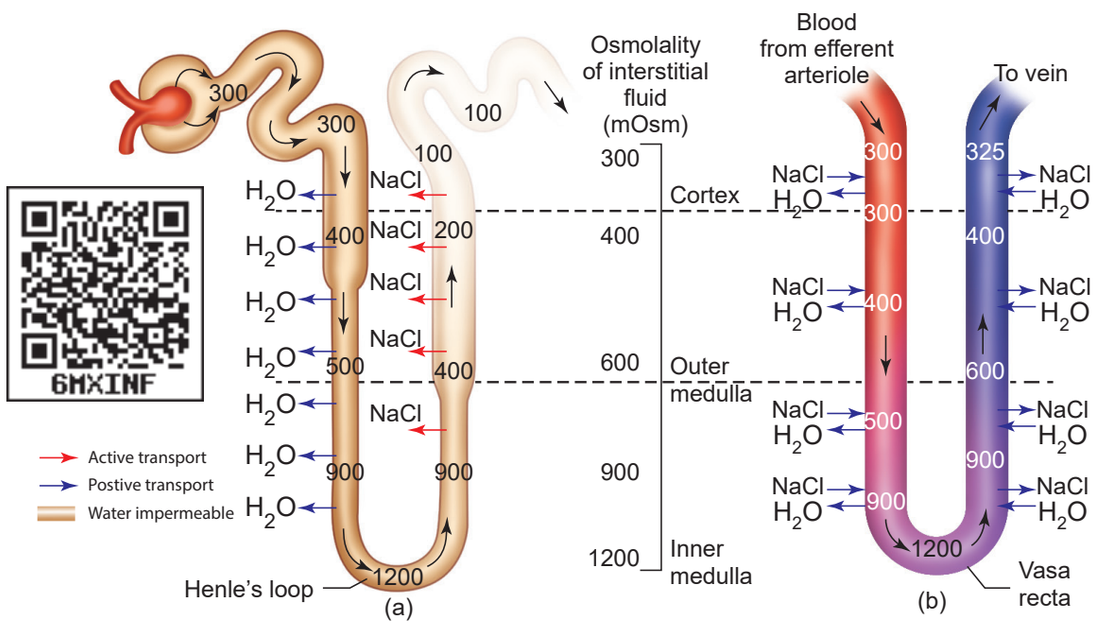


Figure 8.8 (a) Counter current multiplier – the long loops of Henle of the juxtamedullary nephrons create the medullary osmotic gradient (b) Counter current exchanger – Vasa recta preserves the medullary gradient while removing reabsorbed water and solutes.

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